

Patient ID: GarrettPeriod: May 4 to June 1, 2026Report Date: June 2, 2026Coverage: 656/658h (100%)

Patient's monitoring period was less than 30 days; this report covers all available data. Decision-support summary derived from longitudinal vital sign trends; interpret in clinical context.

Clinical Alerting Thresholds

Very Low HR < 40 bpm

Very High HR > 110 bpm

Low HR < 45 bpm

Elevated Breathing > 24 brpmElevated HR > 95 bpmHigh HR > 100 bpm

Last 24 Hours

Last 24 Hours: within normal range			
Vital Stat	Avg	Min	Max
Heart Rate	61 bpm	51 bpm	72 bpm
Breathing	20 brpm	12 brpm	41 brpm

Last 24 hours: no episodic events, vitals within normal range.

Episodic Events (last 24h)

No episodic events in the last 24 hours.

Patient clinical status over 29 days from May 04 to Jun 01

■ HR ■ Breathing

Elevated Breathing May 08 to May 08 (1d)	Elevated Breathing May 13 to May 23 (11d)	Elevated Breathing May 26 to May 28 (3d)
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16 episodic events Detected, spanning 17h total. Conditions: Elevated Breathing. Priority grade: Low

Episodic Events	Total Hours	Highest Burden	Episodes/day	Coverage
16	17h	Elevated Breathing	<1	100%

Major Findings: Stable baseline

Date	Time Span	Duration	Episodes	Condition	Avg	Min	Max	Comment
May 08	6 PM to 7 PM	1h	1	Elevated Breathing	26	18	39	Check SpO2 and lung sounds; assess for fluid overload or early infection
May 13	12 AM to 1 AM	10h	10	Elevated Breathing	25	15	48	Check SpO2 and lung sounds; assess for fluid overload or early infection
May 26	1 AM to 2 AM	6h	5	Elevated Breathing	25	15	42	Check SpO2 and lung sounds; assess for fluid overload or early infection

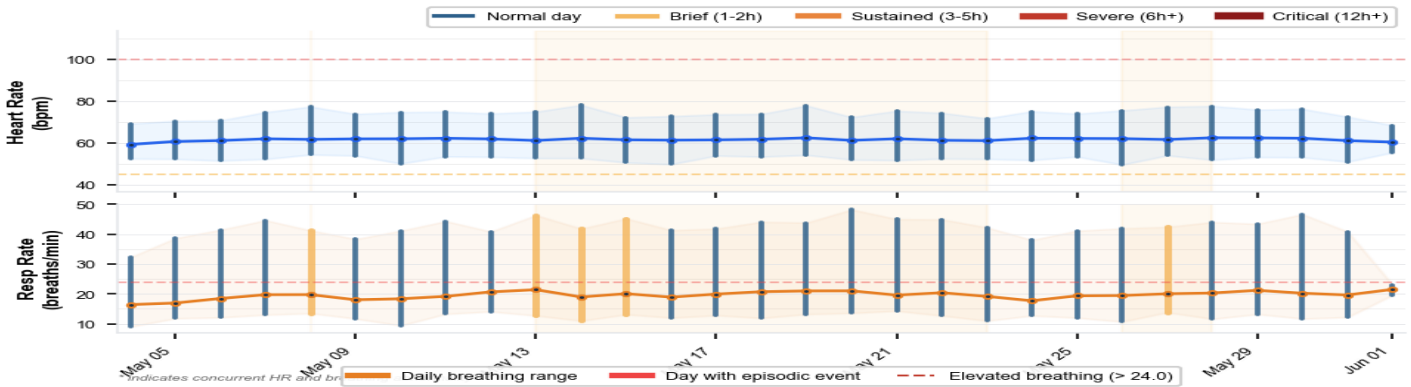
Daily recorded hours declined from 9.0 to 1.0 (89%) over the monitoring period.

Suggested Clinical Review Actions

- Elevated Breathing (avg 25 brpm, peak 48 brpm): Check SpO2 and lung sounds; assess for fluid overload or early infection
- Overall trajectory shows 3 unstable phases with 7 bpm variability. Consider increasing monitoring frequency and lowering escalation threshold.
- Daily recorded hours declined from 9.0 to 1.0 (89%) over the monitoring period.

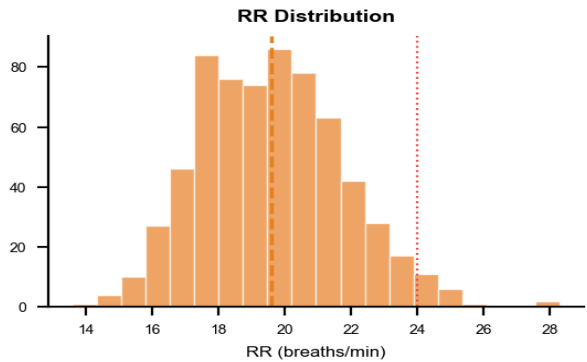
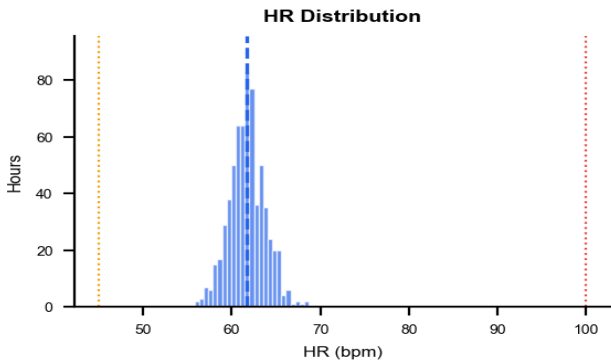
Clinical Pattern Observations:

- 1. **Clustered pattern:** Episodic events on 5 of 29 days (17%).
- 2. **Nocturnal pattern:** 5 of 6 eligible episodes during evening or night hours.

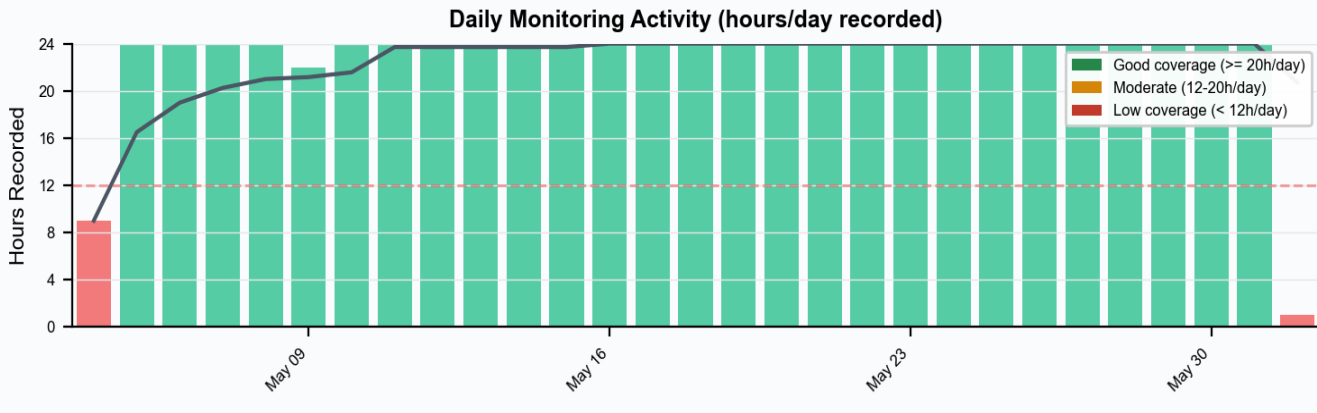


Red bars indicate days with detected episodic events.

Vital Sign Distribution



Monitoring Activity



Bars indicate monitoring hours/day. Red threshold line indicates clinical monitoring baseline.

■ HR episode ■ RR episode ■ Recorded, no episode ■ No data

Episodic event: threshold exceeded continuously for ≥ 1 hour. Episodes separated by ≤ 1 hour of normal vitals are counted as the same event. Brief threshold crossings that do not sustain are not flagged.